

HEALTHCONNECT CLINIC EXPERIENCE LAB

SUMMARY OF MAJOR PROJECT ISSUES ENCOUNTERED

Improving Patient Appointment Attendance and Healthcare Support Using Data and AI

Document Control

Project	HealthConnect Clinic Experience Lab
Project Title	Improving Patient Appointment Attendance and Healthcare Support Using Data and AI
Track	Project Management
Reporting Period	Week 5
Document Type	Summary of Major Project Issues Encountered
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1. ANALYTICAL READINESS CREATED A DOWNSTREAM DEPENDENCY

The transition from planning to execution highlighted the importance of having sufficiently prepared and validated analytical inputs before downstream activities could progress with confidence. Data preparation and exploratory analysis therefore became an important control point for subsequent predictive and intervention-related activities.

Management Response: Project Management maintained visibility of the data-to-analysis dependency and monitored the progression from data preparation through validated analytical outputs and downstream handoff.

Current Position: ● GREEN — CONTROLLED

2. CROSS-WORKSTREAM DEPENDENCIES REQUIRED MORE ACTIVE COORDINATION

As individual tracks progressed, it became evident that several outputs would serve as inputs to other workstreams. In particular, analytical findings may inform predictive modelling and patient-engagement decisions, while later outputs will need to converge within the operational integration and testing stages.

Management Response: Relevant workstream interfaces, expected outputs and handoff conditions were made visible through the Track Coordination Plan, allowing dependencies to be monitored without requiring all tracks to progress simultaneously.

Current Position: ● GREEN — CONTROLLED

3. WORKSTREAM PROGRESS REQUIRED DEPENDENCY-BASED RATHER THAN SEQUENTIAL CONTROL

The project could not be managed effectively as a single linear sequence. Some activities could progress independently, while others required specific upstream outputs before they could become fully enabled.

Management Response: A controlled-parallel approach was maintained, allowing independent work to continue while Project Management monitored activities with genuine upstream dependencies.

Current Position: ● GREEN — CONTROLLED

4. COMPLETION OF WORK DID NOT AUTOMATICALLY ESTABLISH DOWNSTREAM READINESS

During execution, a distinction became necessary between completing an activity and producing an output that is sufficiently reviewed, transferred and usable by another workstream. This is particularly relevant where analytical, predictive, engagement and operational outputs will eventually converge.

Management Response: Project Management incorporated output readiness, handoff, validation and acceptance into cross-track coordination rather than treating task completion alone as evidence of downstream readiness.

Current Position: ● AMBER — ACTIVE CONTROL

5. EMERGING ANALYTICAL FINDINGS REQUIRED FURTHER VALIDATION BEFORE DECISION USE

Data & Analytics

The Data Analytics work produced important observations, including an overall no-show rate and patterns involving previous no-show history, appointment timing, reminder channels and patient distance. However, observed patterns should not automatically be treated as confirmed drivers of no-show behaviour.

48.46% Overall appointment no-show rate	77% No-show rate — patients 30–40 km away who received reminders
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Rather than interpreting the 30–40 km reminder finding directly as reminder failure, further examination is required to understand the interaction between distance, reminder channel and previous no-show behaviour.

Management Response: Project Management flagged these areas for further analytical consideration and ensured that emerging findings are treated as evidence requiring appropriate validation before being used to support downstream project decisions.

Current Position: ● **AMBER — ACTIVE CONTROL**

6. INTEGRATION REQUIREMENTS BECAME MORE DEFINED AS OUTPUTS EMERGED

Process & Operational Integration

As the workstreams moved beyond planning, the relationship between predictive intelligence, patient engagement, Generative AI support and clinic processes became more apparent. This introduced a need to consider how individual outputs would ultimately operate together rather than assessing each workstream in isolation.

Management Response: Process & Operational Integration was maintained as a key receiving interface, with integration implications to be reviewed as downstream outputs mature and become available for validation.

Current Position: ● **AMBER — ACTIVE CONTROL**

MANAGEMENT ASSESSMENT

The issues encountered during Week 5 have been primarily coordination and execution-control challenges rather than project-stopping blockers.

The Most Significant Management Considerations Are

- Maintaining reliable movement of information and outputs between connected workstreams.
- Distinguishing completed activities from outputs that are ready for downstream use.
- Preventing emerging analytical observations from being treated as validated conclusions prematurely.
- Preserving controlled parallel progress while protecting genuine dependencies.
- Preparing the project for increasing integration as the workstreams move toward testing and refinement.

Overall Issue Position

● **AMBER — CONTROLLED EXECUTION**

Project Management Position

No unresolved project-level blocker or formal completion-date variance has been established from the current evidence. The immediate management priority is to maintain dependency visibility, validate emerging outputs and ensure that downstream activities become enabled on the basis of sufficiently reliable inputs.